SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma. · NCCP 00415 · version 5 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

Schedule
Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests, ECG and heart-function monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Dabrafenib
ORAL BRAF INHIBITOR

- ☐ Fever and chills, dehydration
- ☐ Skin rash and new skin tumours
- ☐ Headache, fatigue and joint pain
- ☐ Eye inflammation
- ☐ Bleeding and liver changes
- ☐ Harm to pregnancy and drug interactions

Trametinib
ORAL MEK INHIBITOR

- ☐ Reduced heart pumping function
- ☐ Retinal and other eye problems
- ☐ Lung inflammation
- ☐ Diarrhoea, swelling and rash
- ☐ Bleeding, liver changes and muscle injury
- ☐ Harm to pregnancy

Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00415 v5.

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